INTEGRATED CLINICAL HYPNOTHERAPY FOUNDATION

Formerly California Hypnosis Institute Of India

A Basic Course

In

INTEGRATED CLINICAL HYPNOTHERAPY (THEORY)

LEVEL I

Office: (M) +91 983 340 265 3/55

www.ekaa.co.in
admin@ekaa.co.in

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OBJECTIVES FOR A BASIC COURSE IN

INTEGRATED CLINICAL HYPNOSIS (Theory)

At the end of the basic course, the participants shall:

- understand the theory of mind
- understand the theory of suggestibility
- be able to determine whether a client is physically suggestible or an emotionally suggestible person
- understand why and how an individual gets into a hypnotic state
- be able to understand deepening techniques and awakening techniques

- understand the theory of Self-Hypnosis
- learn to practice Self Hypnosis for Self-Empowerment and excellence
- to be qualified to learn the Practicals of Level 1
- to be qualified to learn the theory of advanced hypnotic techniques and modalities

2 DAY –PROGRAMME SHEET
Day 1

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From	to	Duration in	Contents
900	930	30	Registration & Introduction and objectives
930	1000	30	History of development of human mind
1000	1030	30	Brief history of Hypnosis and use of hypnosis
1030	1045	15	**Tea Break**
1045	1200	75	Theory of Mind
1200	1215	15	What is hypnosis?
1215	1300	30	Theory of Suggestibility
1300	1345	45	**Lunch**
1345	1430	45	Laws of suggestibility
1430	1500	30	suggestibility questionnaire
1500	1530	30	Determining suggestibility of participants and questionnaire debrief
1530	1615	45	Theory for depth of hypnosis, deepening, PHS and awakening
1615	1630	15	Tea Break
1630	1700	30	Clearing client misconceptions and fears and FLIP
1700	1730	30	Wording of Suggestions
1730	1745	15	Theory of Autodual Induction
1745	1800	15	Wrap Up

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2 DAY –PROGRAMME SHEET
Day 2

From	to	Duration in	Contents
900	1000	60	Recap of Day 1
1000	1115	75	Theory & experiential Self-Hypnosis
1115	1130	15	Tea Break
1130	1230	60	Open Book Test
1230	1300	30	Trainer Evaluation with discussion of test Results
1300	1315	15	Student Feedback Online
1315	1330	15	Wrap Up & Closure

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SUGGESTIBILITY QUESTIONNAIRE 1			
1	Have you ever walked in your sleep during your adult life?	YES	NO
2	As a teenager did you feel comfortable expressing your feelings to one or both of your parents or friends?	YES	NO
3	Do you have a tendency to look directly into a person's eyes and/or move closely to them when discussing an interesting subject?	YES	NO
4	Do you feel that most people, when you first meet them, are uncritical of your appearance?	YES	NO
5	In a group situation, with people that you have just met, would you feel comfortable drawing attention to yourself by initiating a conversation?	YES	NO
6	Do you feel comfortable holding hands or hugging someone you are in a relationship with in front of other people?	YES	NO
7	When someone talks about feeling warm physically, do you begin to feel warm also?	YES	NO
8	Do you tend to occasionally tune out when someone is talking to you because you are anxious to come up with your side, and at times not hear what the other person said?	YES	NO
9	Do you feel that you learn and comprehend better by seeing and/or reading than by hearing?	YES	NO
10	In a new class or lecture situation do you usually feel comfortable asking questions in front of the group?	YES	NO
11	When expressing your ideas do you find it important to relate all the details leading up to the subject so the other person can understand it completely?	YES	NO
12	Do you enjoy relating to children?	YES	NO
13	Do you find it easy to be at ease and comfortable with your body movements, even when faced with unfamiliar people and circumstances?	YES	NO
14	Do you prefer reading fiction rather than non-fiction?	YES	NO
15	If you were to imagine sucking on a sour, bitter, juicy, yellow lemon, would your mouth water?	YES	NO
16	If you feel that you deserve to be complimented for something well done, do you feel comfortable if the compliment is given to you in front of other people?	YES	NO
17	Do you feel that you are a good conversationalist?	YES	NO
18	Do you feel comfortable when complimentary attention is drawn to your physical body or appearance?	YES	NO

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SUGGESTIBILITY QUESTIONNAIRE 2			
1	Have you ever awakened in the middle of the night and felt that you could not move your body and/or talk?	YES	NO
2	As a child, did you feel that you were more affected by the tone of voice of your parents than by what they actually said?	YES	NO
3	If someone you are associated with talks about a fear that you too have experienced, do you have a tendency to have an apprehensive or fearful feeling also?	YES	NO
4	After having an argument with someone, do you have a tendency to dwell on what you could or should have said?	YES	NO
5	Do you tend to occasionally tune out when someone is talking to you and do not hear what was said because your mind drifts to something totally unrelated?	YES	NO
6	Do you sometimes desire to be complimented for a job well done, but feel embarrassed or uncomfortable when complimented?	YES	NO
7	Do you often have a fear or dread of not being able to carry on a conversation with someone you just met?	YES	NO
8	Do you feel self-conscious when attention is drawn to your physical body or appearance?	YES	NO
9	If you have a choice, would you rather avoid being around children most of the time?	YES	NO
10	Do you feel that you are not relaxed or loose in body movements, especially when faced with unfamiliar people or circumstances?	YES	NO
11	Do you prefer reading non-fiction rather than fiction?	YES	NO
12	If someone describes a very bitter taste, do you have difficulty experiencing the physical feeling of it?	YES	NO
13	Do you generally feel that you see yourself less favorably than others see you?	YES	NO
14	Do you tend to feel awkward or self-conscious initiating touch (holding hands, kissing, etc.) with someone you are in a relationship with in front of other people?	YES	NO
15	In a new class or lecture situation do you usually feel uncomfortable asking questions in front of the group even though you may desire further explanation?	YES	NO
16	Do you feel uneasy if someone you have just met looks directly in the eyes when talking to you, especially if the conversation is about you?	YES	NO
17	In a group situation with people you have just met, would you feel uncomfortable drawing attention to yourself by initiating a conversation?	YES	NO
18	If you are in a relationship, or are very close to someone, do you find it difficult or embarrassing to verbalize your love for them?	YES	NO

SUGGESTIBILITY

Thisquestionnaireistounderstandhowyoutakeinorinterpretinformation.Asthistraitisfrozen duringteenageandremainsconstantthereafter,mostofthequestionsneedyoutothinkof situationsduringyourteenageandhowyourespondedtothemmostofthetime.Thereisnorightor wronganswer.Iftherehasbeennopracticalexperience,imaginehowyoumighthaveinteracted duringyourteenageinthosesituationsandanswer.Pleaseanswerallthequestionsinbothsheets.

1. Scoring Instructions for Suggestibility questionnaires

a. Count the number of **yes** answers to Questionnaire No.1. Give yourself ten points for each **yes** answer to questions 1 and 2, and five points for each **yes** answer to questions 3-18.
b. Do the same for Questionnaire No.2
c. Add the two scores together to obtain the combined score.
d. Divide the score for questionnaire number 1 by the sum total of both the questionnaires and multiply it by 100. This indicates the percentage of physical suggestibility.
e. Subtract that score from 100 to determine the percentage of emotional suggestibility.

i.e. Physical Suggestibility: A / (A+B) X 100 =_____% (p)
Emotional Suggestibility: 100 – (p) =_____% (e)

2. Indications in suggestibility questionnaire

- "Yes" answers to questionnaire number one are indicative of physical suggestibility as they show physical suggestible behavior.
- "Yes" answers to questionnaire number two are indicative of emotional suggestibility as they show emotional suggestible behavior.

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Chapter 1
History of Development of the Human Mind

1. BODY-MIND CORELATION

In a crisis, the Mind perceives danger and the Body responds by letting the Autonomous Nervous System take over.

There are two divisions of the autonomous nervous system:
Sympathetic: The sympathetic division is activated during the fight-flight mechanism. This is represented by various physiological changes in the system such as palpitation, increased rate of breathing and rush of adrenalin leading to action.

Parasympathetic: The parasympathetic division is a self -regulating, stabilising system that brings us back to a state of balance, i.e. homeostasis.

During sympathetic arousal, physiological changes occur, preparing us for fight -flight. After the danger has passed, the parasympathetic system restores balance or homeostasis.

2. INHIBITION

Definition: Unconscious defence against forbidden, unacceptable learned conditions, instinctual drives or impulses.

Originally, primitive man functioned completely by instinct. He did not have inhibitions. With the passage of time, man started developing the ability to think. Amongst the first things that man thought about was survival. As a fall out of this, “rules for living” were formulated. With the formation of rules, inhibitions started developing. Some of the early inhibitions that developed were modesty, privacy and sexual taboos.

Man came to the increased realization that he did not have to act on every impulse that confronted him. He then started to develop **conscious control.**

At this point, the fight mechanism started to become anxiety and he started to feel it in his body.

The flight mechanism also developed a modern way of coping by going into freeze mode by going into freeze mode.

Chapter 2
Hypnosis

1. DEFINITION OF HYPNOSIS

Hypnosis is a state of altered awareness in which access is available to the sub -conscious mind. The process is initiated by an overload of message units, leading to break down of the inhibitory processes of the critical mind. This in turn triggers the fight-flight mechanism and ultimately results in a hyper-suggestible state, providing access to the subconscious mind.

Unlike the "sleep state" in which you are oblivious of what is happening around you, the hypnotic state is one in which the conscious mind is awake and aware of what is going on, simultaneously providing access to the sub-conscious mind.

In the state of Hypnosis – it is possible to learn.

2. CREATING THE STATE OF HYPNOSIS

Hypnosis involves a state of anxiety being created by the hypnotist which in turn triggers off the flight -fight response from the sub -conscious mind. The triggering of the fight mechanism involves the following physiological changes:

a. Changes in breathing
b. Dryness of lips and throat
c. Rapid eye movement

Once in hypnosis, the flight mechanism takes over, breathing becomes shallow, and the body becomes more relaxed.

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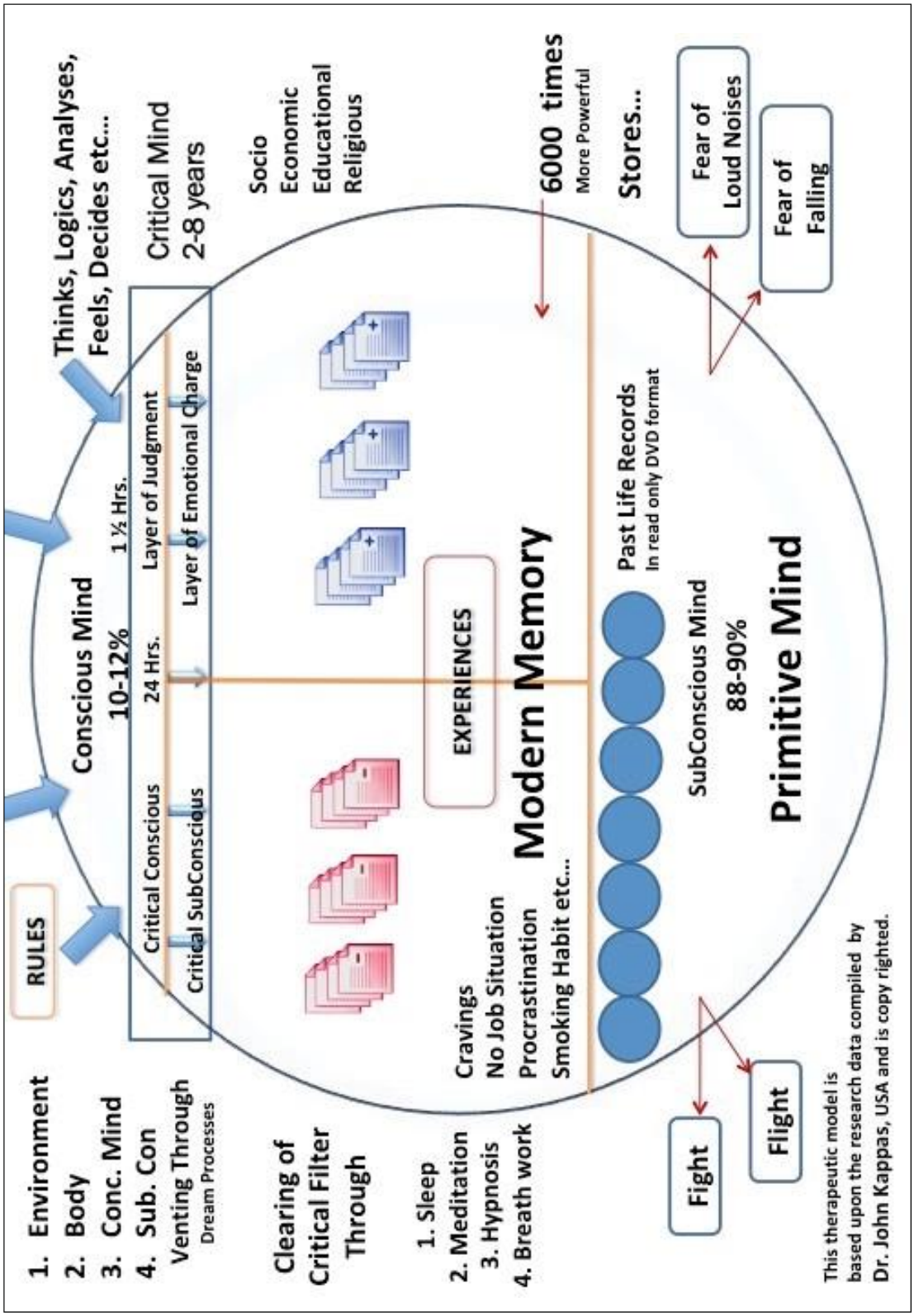
Chapter 3
Theory Of Mind



Chapter 4
Theory of suggestibility

1. SUGGESTIBILITY

Suggestibility can be defined as the manner in which an individual receives and interprets input (i.e. message units) or how the individual was communicated with, during childhood.

Prior to 1967, hypnotists used one of three basic approaches to induce the hypnotic sleep. These were:

1. Eye Fascination
2. Authoritarian Approach
3. Progressive Relaxation

Unfortunately, most hypnotists still use direct, literal words for induction as well as formulation of suggestions. This did not and still does not work with 50% of the population because direct, literal suggestions only work with one type of individual.

Dr. Kappas found that there are two types of suggestibility. This explained why the direct, literal suggestions were not working with half the population.

2. EMOTIONAL SUGGESTIBILITY

Definition: A suggestible behaviour characterised by a high degree of responsiveness to inferred suggestions affecting emotions but would not be able to grasp suggestions affecting the physical body or physical movements. An emotional suggestible person learns more by inference than by direct literal suggestions.

There is a natural tendency to enter hypnosis at the hypnoidal depth.
An emotional suggestible is left-brain dominant.

3. PHYSICAL SUGGESTIBILITY

Definition: A suggestible behaviour characterised by a high degree of responsiveness to suggestions affecting the physical body, but will not relate to suggestions affecting his emotional behaviour.

They naturally enter hypnosis at cataleptic stages or deeper.
A physical suggestible is right brain dominant.

4. SOMNAMBULIST

A somnambulist is an individual who has 50/50 suggestibility (i.e. 50% physical suggestible, 50% emotional suggestible). This type will respond equally well to all suggestions, both direct and inferred, affecting both the body and the emotions.

5. INTELLECTUAL SUGGESTIBLE

This type of person is very analytical, generally a high emotional suggestible. The intellectual suggestible fears being controlled by the hypnotist and is constantly trying to analyse, reject, or rationalise everything the hypnotist says.

The induction of choice for this type of suggestibility is the "Auto-dual" method.

6. HOW WE COMMUNICATE

The physical suggestible individual learns literally and directly, but speaks in metaphors and inferences. The emotional suggestible learns indirectly and inferentially, but speaks directly and literally.

Example: *Would you tell me your name?*
The Physical would respond **Yes.**
The Emotional would respond by telling you their name.

7. HOW SUGGESTIBILITY IS LEARNED

Initially, learning is accomplished through our primary caretaker (i.e. mother), from birth to approximately age five. From ages six to nine, our peers, teachers, and others also influence our suggestibility. From ages nine to fourteen, the secondary caretaker (i.e. father) begins to influence our suggestibility.

Every interpersonal message is made up of three parts:
1. Verbal content.
2. Extra-verbal (non-spoken) content.
3. The state of mind of the receiver.

If mother follows through with what she says to the child, the child learns direct, literal communication (i.e. physical).

If mother does **not** follow through with what she says, the child receives incongruent messages and begins to look for the hidden meaning in what is being said (i.e. emotional) and starts analyzing.
If mother disciplines (e.g. spanks) the child and then rewards (e.g. hugs, kisses, etc.) the inconsistent behaviour of the mother compels the child to start analyzing the true intent of the mother; thus, moving towards emotional suggestibility.

The physical suggestibles place their physical body out front and protect their emotions, particularly rejection. The emotional suggestibles place defensive emotions out front (e.g. fear, embarrassment, discomfort) to protect their physical body.

Chapter 5
Laws of Suggestibility

1. DOMINANT LAWS OF SUGGESTIBILITY

There are five basic laws of suggestibility, the proper application of which will allow the hypnotist to utilize man's natural suggestibility to its fullest. It is man's propensity toward the emotions of fear and greed that make him so susceptible to the influence of these laws.

Fear suppresses our ability to make a decision. As a result any decision made for us during the fear reaction becomes the road of least resistance and is readily accepted by the subconscious mind.
Greed brings about a state of urgency and causes us to react without logical thinking.

Keep these two emotions in mind when referring to the Laws of Suggestibility. The Laws are:

1. **The Law of Reverse Action.**
2. **The Law of Repetition.**
3. **The Law of Dominance.**
4. **The Law of Delayed Action.**
5. **The Law of Association.**

The most common law is the **Law of Reverse Action,** sometimes referred to as the Reverse Psychology. A subject will respond to the stronger part of a suggestion if the alternative prese nted is considerably weaker. For example, in the case of an eye challenge, "Your eyes are sealed shut and you CANNOT open them and the harder you try, the more difficult it becomes."

The Law of Repetition is represented in the fact that the more we do something, the better we become at it. All learning skills follow this law. By repeating suggestions in hypnosis, the stronger the suggestive idea becomes. The effect of hypnosis gets better with every session.

The Law of Dominance governs our entire social structure. Suggestions are taken in far more easily by the mind, when they are seen emanating from a dominant authority.
An example of this law is the suggestion of **deep sleep.** These words spoken in an authoritative tone represent a command to the subject who is apt to accept a suggestion from an authority figure (i.e. the hypnotist).

The Law of Delayed Action says that if there is an idea you put in associated with a triggering mechanism – the idea will take control whenever the trigger is initiated.

The Law of Association simply put is that whenever we repeatedly respond to a particular stimulus in the presence of another, we will soon begin to associate the one with the other. Whenever either stimulus is present, the other is recalled. The post suggestion to re-hypnosis works under this law.

2. CONVERSION TO HYPNOSIS

A suggestibility test may be used to convert a subject into hypnosis.

The purpose of a suggestibility test is:

a. To determine the degree of physical or emotional suggestibility.
b. To formulate suggestions using suitable words so that they may be absorbed at the client's level of control.

The idea is that during the suggestibility test the hypnotist is building up message units.

The hypnotist should begin all suggestibility tests by saying "Now, I am going to test your suggestibility. You cannot pass or fail this test, so don't help or hinder me in any way."

Chapter 6
Theory of Depth of Hypnosis

1. DEPTH

Hypnotic depth has different connotations for different suggestible subjects. As depth is essentially the loss of the grip of the conscious mind on the control that it exercises, hypnoidal, cataleptic and somnambulistic stages for emotional subjects are much different than those for physical suggestibles.

Depth assumes significance because the responsiveness of a client to a suggestion; is a function of his depth. The intensity with which a client would respond to a given suggestion would directly depend on the suggestion being given when he is in his utmost receptive state.

There are three basic levels of hypnosis:

Hypnoidal – Characterized by rapid eye movement (REM). It is the lightest stage of hypnotic sleep.

Cataleptic – Characterized by the eyes moving from side to side. It is the medium stage of sleep.

Somnambulism – Characterized by the eyes rolling up underneath the eyelids. This is the deepest stage of hypnotic sleep.

There are three stages of Somnambulism:

- A first stage somnambulist will exhibit between 20% and 40% spontaneous amnesia.
- A second stage somnambulist will exhibit approximately 60% spontaneous amnesia.
- A third stage somnambulist (the deepest subject) will respond to all types of suggestions. This subject will exhibit 100% spontaneous amnesia; he will remember nothing that occurred while in hypnosis.

Therapy needs to be done at the somnambulistic state for it to be completely effective.

2. SCALE OF IMAGINATION

The goal during the first session is to build enough message units so that the subject is taken to either the first, second, or third stage of somnambulism. It is important during this first session to peak the subject's suggestibility so that a full conversion is obtained. The subject later will automatically revert back to their normal suggestible state. However, the hypnotist can later induce somnambulism if necessary.

3. DYNAMICS OF HYPNOSIS

The basic idea is to influence the subject to respond to your suggestions. You are constantly testing the subject to determine how and why the subject responds in a certain way.

The very first thing s you want to have happen when testing a subject is to have the subject follow your directions. By getting them to respond positively to conscious input, the subject will be better able to respond positively to unconscious input.

While talking to the subject, you are creating an overload mechanism in the subject's mind. They are building expectation and therefore creating greater overload.

An important point is to know whether or not the subject can visualize. If the subject cannot visualize, have them imagine or pretend.

By pointing out the physiological changes, the hypnotist is creating more "yes" or positive messages.

By simulating somnambulism in the induction (i.e. eyeballs rolling up underneath the eyelids), you can artificially create the somnambulistic state. In other words, you are putting the body to sleep; keeping the mind awake.

If the subject opens his eyes during the induction this will increase the number of message units by increasing the stimuli of visual sensations.

During the ar m raising induction, the fight/flight mechanism is used. While the subject's hand and arm are moving toward the face you can, by telling the subject that this is happening, cause the subject to fight it, which will create even more anxiety, until the hand touches the face and the subject retreats into hypnosis.

When doing a challenge, test the challenge first so that you know the challenge will work. Do not challenge a subject if the challenge is going to fail.

At each stage of hypnosis, add a post suggestion to re-hypnosis. This will allow you to bring the subject back to that level of hypnosis.

Paradox – A reversal (e.g. The harder you try, the more difficult it becomes.) is the strongest suggestion you can give, next to the post-suggestion to re-hypnosis.

Power Words – When you want to overload the subject, use power words such as lifting, rising, higher, jerking, etc.

4. DEEPENING TECHNIQUES

At least one, and sometimes all of the following techniques must be employed effectively to increase a subject's hypnotic depth:

1. Establishing rapport
2. Removing fear of loss of control
3. Increasing time spent in the hypnotic state
4. Successfully challenging the subject
5. Using rapid reactional hypnosis
6. Using misdirection

Building rapport, or establishing the atmosphere of the therapeutic relationship, is completely the responsibility of the therapist. A display of sound self -confidence in the therapeutic process is essential for developing trust and positive expectation in the client's mind. However, it is particularly important not to display an attitude of egotism or dominance because so many of the myths surrounding hypnosis include ideas of dominance and control. Never make ridiculous statements such as, *I control your mind. You are under my power, or you will obey my suggestions.*

Never tell a subject that he cannot hear sounds around him as this will only draw his attention to them when prior to such a suggestion he may have not even noticed them. Instead, utilize distracting sounds to your advantage by suggesting that:

> *These outside sounds are the sounds of everyday living. They will not distract or disturb you, but will tend to relax you further.*

5. CHALLENGES

When doing a challenge, do it in the form of a reversal rather than in a direct, literal form.

The challenge is used as a deepening technique. It is <u>not</u> used to "prove" to the subject that they are indeed in hypnosis.

6. REACTIONAL HYPNOSIS

Definition: Repeatedly awakening the subject and re-hypnotizing this person with a post - suggestion to re-hypnosis; is an effective method of achieving depth in hypnosis.

Script of Reactional Hypnosis:

Tell:
You are listening to my voice. Any other sounds that you hear in the background will help you to focus on the sound of my voice only.
(Pause)
I now want you to very slowly and very gently open your eyes.

Tell: *When I say **1** keep your eyes open and when I say **2**, just shut your eyes.*

1, 2, 2, 1, 1, 1, 2, 2… And so on.

Note: Alter the speed and sequence of numbers periodically, thus confusing the client and adding pressure to his eye muscles. When you observe the clients' eyes tiring and beginning to make mistakes then; s nap your fingers, touch the third eye and say '*Deep Sleep*', gently asking the client to shut his eyes.

7. PROGRESSIVE RELAXATION

Progressive relaxation techniques are usually used as deepening techniques after the hypnotic state has been reached.

Script:

Give suggestion:
Turn your attention, your concentration to the toes of both your feet . (pause)

Now let your concentration move from yo ur toes, into your heels, (pause) *…into your ankles and then to the calves of your legs;* (pause) *and from your toes to your knees feel a wave of relaxation.*

Now feel the relaxation moving up through your thighs, your hips, and in through the stomach muscles. (pause)
Now the relaxation will move up into the solar plexus, across the chest area, (pause) *in through your shoulders and down through your arms, your hands and fingers.* (pause)

Simultaneously, relax the muscles of your back…the lower back (pause), the middle back (pause), the area of your upper back (pause)…right up to your shoulders. (pause)

And the neck muscles are relaxing. (pause) *The relaxation moves up through the scalp, across the forehead, and down over the eyelids;* (pause) *the facial muscles and jaw muscles, and the physical body relaxing deeply and completely with every breath.* (pause)

Now, as I count from ***five down to zero****, each count prepares you to enter the hypnotic sleep more deeply at zero.* (pause)
And now five, four, three, two, one, and zero **touch third eye** *and say 'Deep Sleep', and let's go deeper now.*

Wait for a while.

8. POST HYPNOTIC SUGGESTION

Post Suggestion to Re-Hypnosis:

Any suggestion given to a client under hypnosis which will affect his or her response/behaviour in a desired manner after the hypnotic session is over is called a post-hypnotic suggestion.
The post suggestion to re-hypnosis works under the law of association.

The two most common forms of post hypnotic suggestions are:

1) Post-hypnotic suggestion to re -hypnosis, which leaves no room for doubt about the client re - entering hypnosis at another time.

Following is a generalised post suggestion:

Each and every time	Covers the present and future
*I suggest '**deep sleep'** to you*	*I* and *you* are exaggerated
you will sleep	All are exaggerated
Quickly soundly and deeply	Exaggerate *deeply*, specially with a deep subject
and the physical body will relax	The subject is more prone to accept the post suggestion, because this is beneficial and eliminates any possibility of threat.

2) Anchors and triggers.

Anchors: can be natural

Triggers: need to be consciously activated

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Chapter 7
Initial Interaction with the Client

1. PATTERN OF THERAPY

All hypnotherapy follows the same basic pattern. First, the client is allowed to guide the discussion. During this initial phase, the client generally does not vent negative emotions. Second, in the hypnosis state, all the positive suggestions for change are placed in the subconscious mind, which allows the client to feel better.

2. PRE-INDUCTION

Definition: an introduction to hypnosis to prepare the subject for the induction. It should include an explanation of hypnosis and an idea of what the subject can expect to experience in the state.
There are two main purposes to the pre-induction:
1. Dispel misconceptions and fears- explain the theory of mind.
2. Build expectation and rapport.

Never use the word cure. It implies a medical treatment. Hypnotists treat many things but never claim to cure anything.

3. RAPPORT

Twenty-five percent of the population is naturally suggestible to you at anytime therefore; you will automatically have rapport with them. This is perhaps one of the most important aspects of therapy. It is the clients' belief system that you can help them which is what ultimately results in the clients' behaviour changing.

4. FAIT ACCOMPLI

Fait accompli is the concept that if you pretend and go along with the hypnotist, you will end up in the hypnotic state. **The pretense breaks the defense.**
As a hypnotist, you can use thi s by telling the subject to just "pretend to go along with this." This approach works well with emotional subjects.

5. HANDWRITING

Handwriting is a sub-conscious process. You are what you write. What you change in the handwriting is what you will become. Handwriting analysis is the quickest approach to diagnosing emotional and physical suggestibility and sexuality.

6. FIRST SESSION

The first session is the most important session of the entire therapy. Most therapists lose at least 50% of their clients after the first session. Therefore, it is important to have the client know that you understand them and their problem. This is where rapport is of great help.

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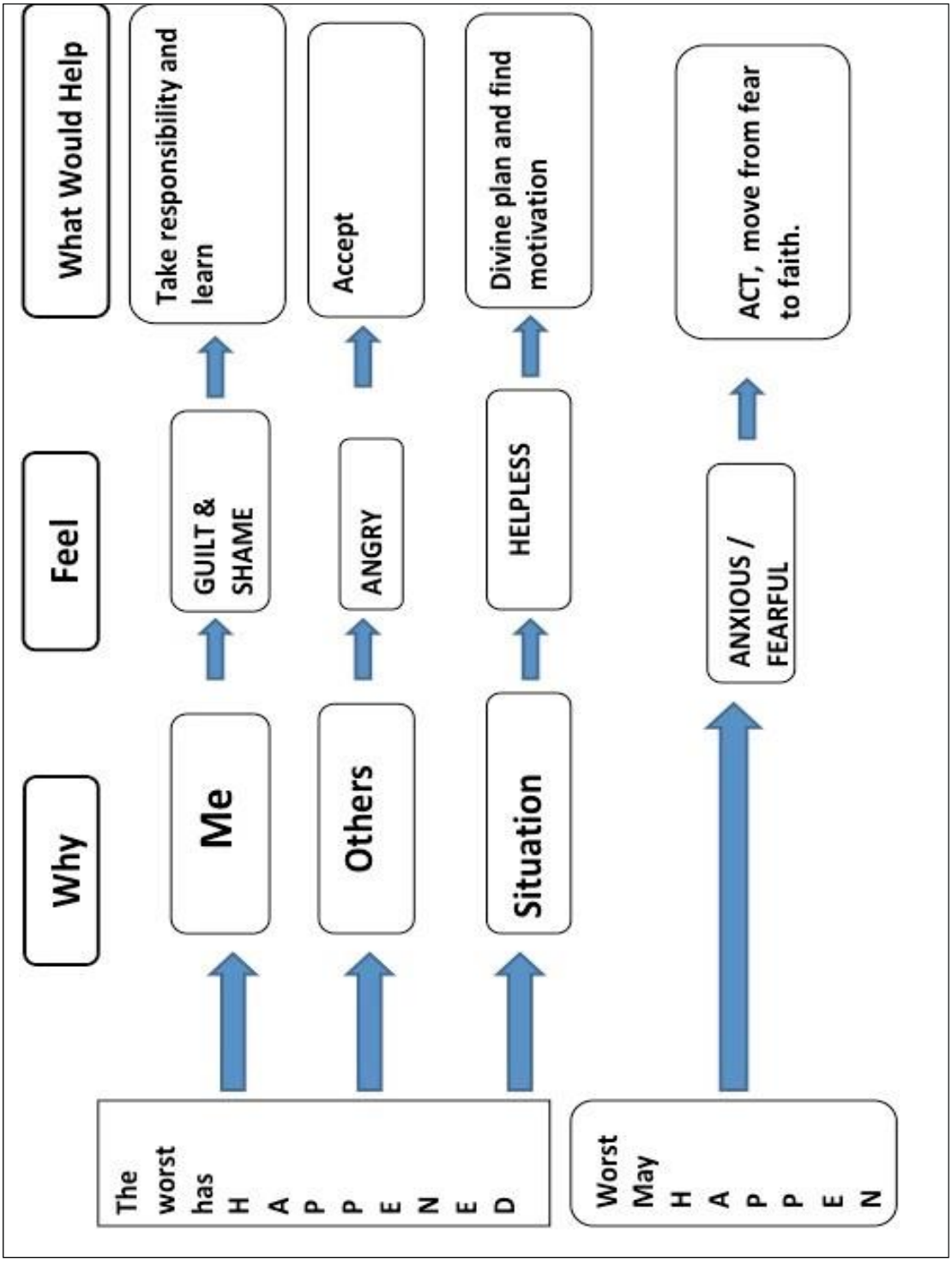
Chapter 8
FLIP



Chapter 9
Wording of Suggestions

1. DYNAMICS OF THE SUGGESTIONS

In the case of a physical suggestible, message units are taken in literally as a thought. The thought then becomes an image, then a physical feeling, and finally an emotional response.

An Emotional suggestible takes in message units as a thought. The thought then becomes an image, then an emotional feeling and finally a physical reaction.

While giving suggestions to a Physical, work on the conflict literally. With an Emotional, work on what they are deprived of by the conflict. The clue to suggestions with an emotional is finding out how they feel.

It is important to let the emotional deduce the effect of the suggestion themselves.

During hypnosis, the critical area of the mind is in a state of abeyance. It does not have the chance or the desire to fight the suggestions and suggestive ideas entering the subconscious mind.

Suggestibility is testable! Any attempt to place a person in the state of hypnosis without testing their suggestibility may be setting yourself up for failure.

Chapter 10
Auto Dual Induction

Perhaps, one of the most difficult things for the human mind is to give up control. This is particularly true, especially in the case of a high emotional or intellectual suggestible client. Very often the fear of being controlled by the hypnotherapist results in the subject trying to analyze himself out of hypnosis or challenging the premise that he has been hypnotized, even though he himself has taken the trouble to fix the appointment and is paying for the session as well.

The *Auto Dual method* is an excellent induction technique which prevents such subjects from analyzing what the therapist is doing; thereby causing him to enter a hypnotic state through a form of misdirection. It allows the client to believe that he is hypnotizing himself instead of being hypnotized by the therapist which gives him the assurance that he is still in control. The therapist here is actually creating an illusion in the client's mind, that he is hypnotizing himself.

This is the preferred route of induction of an intellectual suggestible client.

Chapter 11
Self Hypnosis

1. SELF HYPNOSIS

The basic difference between hetero -hypnosis and self -hypnosis is that hetero -hypnosis involves the disorganization of the inhibitory processes, whereas self -hypnosis requires the organization of the inhibitory processes.

Two laws that govern self-hypnosis are:

- **The Law of Repetition –** By doing it over and over, you become better at it.
- **The Law of Association –** The idea of associating to the state and to the keys that you use.

2. TREATMENT AND SELF HYPNOSIS

Self-hypnosis is not used to "cure" a problem. You do not treat yourself. It m ay be used effectively for relaxation, stress control, and other non-therapeutic reasons such as general self-improvement or sports improvement.

3. MYTHS SURROUNDING SELF HYPNOSIS

Anytime a tape is used, or someone else's words (e.g. from a book) are us ed, the hypnosis is hetero -hypnosis rather than true self-hypnosis. Someone else is still hypnotizing the subject.

4. SELF HYPNOSIS KEYS

These are words that you relate to, both physically and emotionally, that will work the best to trigger the self-hypnosis state.

Physical Keys – Words that you respond or relate to physically.

- Floating,
- Heavy,
- Lightness
- Limp,
- Loose,
- Relaxation, or
- Tingling

Emotional Keys – A word that you respond or relate to emotionally.

- Calmness,
- Confidence,
- Contentment,
- Happy,
- Joy,
- Peace, or
- Success

Intellectual Key is Deep Sleep and is universal for everyone.

When this technique is used either for meditation or to ensure a sound sleep, replace the word "deep sleep" with the word "meditate". In such cases counting up is not required as the person will ideally go into biological sleep and wake up through the natural process.

5. TIME DISTORTION

When in hypnosis, time distortion can take place. It may manifest as either expansion or contraction. Twenty minutes may seem as short as five or as long as an hour.

6. SELF-HYPNOSIS NOTES

Never go into self -hypnosis with a strong negative attitude. When you enter your sub - conscious mind with a positive suggestion you ensure that you are not taking in the negatives in with you.

While formulating suggestions, do not use negative words or feelings such as "I will not" or "I cannot." Always phrase all suggestions in a positive manner and as if they have already happened / are happening.

Also, do not work on many suggestions all at once. Work only one or two suggestions at a time, allowing the law of repetition to work.

7. LAW OF ASSOCIATION AND SELF -HYPNOSIS

In the first session the facilitator through a modality introduces the client to a hypnotic state. He then gui des the client to associate that state with key words or triggers. By the law of association, subsequently whenever the client invokes the triggers, he automatically allows himself to enter the hypnotic state for suggestion reinforcement.

Group session for Self – Hypnosis

PROCEDURE FOR SELF HYPNOSIS

Important:

- Avoid practicing self -hypnosis when lying down in bed or when very tired or sleepy.
- Repeat this process separately for every suggestion.
- Repeat the self-hypnosis process at the same place to build an association of the place and time, just like one would for meditation.

Step	ACTIVITY
1	Pick a suggestion mentally or write it down before hand. · keep the sentence short · word the suggestion in a + ve manner/ language.
2	Preparation: · Take off shoes and socks (if worn). · No tight clothes-loosen tight clothes. · Remove wrist watch, put things down, if held in the hand. · Turn off your cell phones, pagers or any other device.
3	a. Sit comfortably on a chair with feet flat on the floor, b. Put hands on the thighs. c. Become comfortable, and recline. or d. If you lie down, do not lie in the position you sleep in, keep your head at least 1 foot (33 cms.) higher than your feet.
4	Take three deep breaths · Breathe in through the nose- inhale positive energy, · Breathe out through the mouth- exhale negative energy.
5	With the eyes comfortably closed, breathe normally · feel your breath coming in and going out.
6	After a few minutes, whenever you feel comfortable · Repeat the **physical key word** 7 times (Get in touch with the feeling of the physical key word, get in touch with yourself).
7	**WAIT FOR 30 SECONDS** .
8	Focus on your breath- feel your breath coming in and going out, After sometime, whenever you feel comfortable · Repeat the **emotional key word 7 times** (Get in touch with the emotional feeling of the key word, get in touch with yourself).

9	**WAIT FOR 30 S ECONDS**.
10	Focus on your breath- feel your breath coming in and going out, After sometime, whenever you feel comfortable • Repeat '**deep sleep' 7 times**. • Feel it mentally.
11	**WAIT FOR 30 SECONDS**.
12	Concentrate on the breath -feel your breath coming in and going out, After sometime, whenever you feel comfortable • Repeat **'your suggestion' 21 times**
13	**WAIT FOR 30 SECONDS**
14	Concentrate on the breath -feel your breath coming in and going out, After sometime, whenever you feel comfortable • Tell yourself you are going to bring yourself out of hypnosis on the count of 1-5
15	Count 1, 2, 3, 4, 5. Repeat to yourself "Eyes Open, Wide Awake"
16	Open your eyes- change position - ensure that you come out of self-hypnosis.
17	Repeat all steps for each **NEW** suggestion you want to give yourself.

APPENDAGES

Some of the ways in which Hypnotherapy can help:

SR.	AILMENT	SR.	AILMENT
1	Abandonment Issues	2	Age regression
3	Abuse	4	Aggression
5	Acidity	6	Addictions
7	Agoraphobia	8	Amnesia
9	Anesthesia	10	Anxiety
11	Anger	12	Anorexia
13	Assertiveness	14	Attention issues
15	Attitude Issues	16	Asthma
17	Back pain	18	Bed wetting
19	Behavioral issues	20	Blood pressure
21	Bulimia	22	Burns
23	Career issues	24	Cerebral Palsy
25	Changing habits	26	Chemotherapy support
27	Child birth issues	28	Chronic pain
29	Communication issues	30	Concentration improvement
31	Cramps	32	Cravings
33	Creativity Enhancement	34	Dandruff
35	Dealing with Death or Loss	36	Depression
37	Divorce issues	38	Dreams
39	Dyslexia	40	Ear, Nose and Throat issues
41	Eczema	42	Egg Allergy
43	Emotional Disturbances	44	Exam Anxiety
45	Energy drain	46	Enhancing skills
47	Fear of Animals	48	Fear of Darkness
49	Fear of Death	50	Fear of failure
51	Fear of Flying	52	Fear of Heights
53	Fear of Loss of control	54	Fear of Medicines
55	Fear of School	56	Fear of Success
57	Fear of Surgery	58	Fear of Water
59	Fear of Injections	60	Fear of Intimacy
61	Forgiveness	62	Frustration
63	Fructose Intolerance	64	Gambling Addictions
65	Goal accomplishments	66	Guilt
67	Hair fall and other hair issues	68	Headaches
69	Helplessness	70	Hiccups
71	Hopelessness	72	Hostility
73	Hot Flashes	74	Hyperactivity
75	Hypertension	76	Hypochondria
77	Immune System Issues	78	Impotency

SR.	AILMENT	SR.	AILMENT
79	Improving Health	80	Improving sales and marketing
81	Indecisiveness	82	Infections
83	Inferiority complex	84	Inhibition
85	Insecurity	86	Insomnia
87	Irrational thought management	88	IBS (irritable bowel syndrome)
89	Irritability	90	Jealousy
91	Kidney issues	92	Lack of ambition
93	Lack of enthusiasm	94	Lack of direction
95	Lack of initiative	96	Lactose intolerance
97	Low Blood Pressure	98	Medication side effects
99	Memory issues	100	Menstrual aches and pains
101	Mistrust	102	Mood swings
103	Motivation	104	Nail biting
105	Nausea	106	Negativism
107	Nightmares	108	OCD (obsessive compulsive disorder)
109	Overeating	110	Pain management
111	Panic attacks	112	Paralysis
113	Parkinson's Disease	114	Passive Aggressive Behavior
115	Perfectionism	116	Performance Anxiety
117	Pessimism	118	Phobias
119	Polycystic Ovarian disorder (PCOD)	120	Pre and post surgical issues
121	Premature ejaculation	122	Procrastination
123	Prostrate issues	124	Psoriasis
125	Public speaking	126	Reach goals
127	Rejection	128	Relationship issues
129	Relaxation	130	Repressions
131	Resistances	132	Responsibility issues
133	Restlessness	134	Sadness
135	Self-awareness	136	Self-blame
137	Self-confidence	138	Self-control issues
139	Self-criticism	140	Self-defeatism
141	Self-esteem issues	142	Self-expression
143	Self-forgiveness	144	Self-hypnosis
145	Self-image issues	146	Self-mastery
147	Sexual problems	148	Shame
149	Skin problems	150	Sleep disorders
151	Smoking cessation	152	Snoring
153	Social phobia	154	Spondylosis
155	Sports enhancement	156	Stage fright
157	Stress management	158	Stubbornness
159	Study habits	160	Stammering
161	Stuttering	162	Substance Abuse
163	Post surgical recovery	164	Swallowing issues
165	Sweating	166	Thumb sucking

SR.	AILMENT	SR.	AILMENT
167	Thyriod sucking	168	Tics
169	Trauma management	170	Ulcers
171	Unexplained Aches and pains	172	Uterine disorders
173	Vertigo	174	Victimization
175	Warts	176	Weight management
177	Withdrawal symptom management	178	Worry
179	Writers Block	180	

EKAA
A Basic Course in Integrated Clinical Hypnotherapy

Students Section

Students will receive a personal identity card with a student registration number.
This card needs to be carried and/or number quoted for all future workshops you attend.

Future courses with EKAA

Level II: Integrated Hypnotic Modalities for Behavioral Resolutions (5 Days)

Topics covered:-
Ericksonian Hypnosis, Body Syndromes, Emotional Freedom Techniques (EFT), Diagnostic tools, Age Regression (diagnosis and therapy), Pain Management, Fear and Phobia Management, Introduction to Graphology and NLP, etc.

Level III: Advanced Integrated Hypnotic Modalities for Health Resolutions (5 Days)

Topics covered:-
Relationship Management, Family Systems, Crisis Management, Passive Aggressive Behavior, Defence Mechanisms, Past Life Regression, Mental Bank, etc.

Level IV: Integrated Spiritual Hypnosis (6 Days)

Topics covered:-
Past Life Regression Therapy, Parallel Universe Concept for therapy, Life Script, Life between Lives, etc.

Level V: Integrated Clinical Hypnotherapy (8 Days)

Topics covered:-
Foreign Energies and Dark Force Entities - types and release, Surrogate DFE release, Journey of Consciousness, Basic Chakra diagnosis, Metaphysical Aspects of Disease , Smoking Cessation, Complete Body work – Rhythmic Relaxation Technique, Allergy Management, Case Documentation, and lots more.

***Kindly note the above curriculum stands valid with effect from 01st April, 2014 and is subject to change at the discretion of the Institute.**
*** Kindly confirm with the Institute for any clarifications / queries / course fees and registrations.**

Code of Conduct for all Level 1 Students/Therapists

1. We are primarily learning Integrated Clinical Hypnotherapy for Self -focus and Self-Empowerment.

2. We are the Lords and Masters of our Universe. Our Universe is a manifestation / projection, consciously or unconsciously of ourselves.

3. Therapy starts with ourselves and we do not blame others or situations for what we manifest in our Universe.

4. We will only attract the clients that we are ready for and are eligible to handle.

5. We inform clients adequately about the therapy and its limits.

6. We do not misrepresent our professional qualifications, affiliations, and purposes. We disclose , upon request, our training, qualifications, and competencies to prospective clients.

7. We do not reject clients on the basis of gender, race, ethnic background, culture or sexual preference.

8. Our reception and our therapy room are appropriate. Our personal hygiene and the hygiene of our therapy room are impeccable.

9. We are responsible for the therapy, but never act against the best interest of our clients, or against their express will. However, we cannot shirk responsibility by claiming to follow a request from a client. The welfare of the client is the prime consideration at all times. When the client's desires and our professional responsibility conflict, we will end the therapeutic relationship courteously. We do neither blame the client nor ourselves for that.

10. As we deal with intimate thoughts, feelings and details of our client's life, we retain a professional distance. We present our work and ourselves professionally. We do not behave as personal friends or acquaintances. We do not engage in intimate relationships of any kind with clients.

11. We recognize transference, counter– transference and other projections, and seek to restrain these. We don't assume the role of Helper or Savior; we discourage admiration and deference from the client. We avoid making clients dependent on us; we stimulate their own responsibility and independence.

12. We do not try to convert clients to any belief or practice. We respect their convictions, though we may point out how these seem to relate to their problems, if this seems the case.

13. We build a relationship of trust with the client and will not betray that trust. We will treat all information from clients confidentially and will not divulge it without the full consent of the client, except when obliged by law to do so. In publications we protect the anonymity of the client or ask written permission.

14. We only perform those therapies for which we are qualified. Unless otherwise licensed to do so, we do not provide or offer to provide diagnosis or treatment which is limited by law to appropriately licensed practitioners.

15. We stay alert if clients need anything outside our own experience or training. We then refer to colleagues or suggest other suitable methods outside our field.

16. We advise clients to see a doctor when problems could have medical cause. We never suggest decreasing or stopping medication. We limit ourselves to psychosomatic diseases and physical complaints for which no physical cause has been found. We do not pretend to heal serious and terminal diseases, but at best to influence favorably the course of such dis eases.

17. Sessions are only recorded with the consent of the client. The client may withdraw his consent rights after the session. Clients are entitled to copies of records of sessions.

18. We refrain from making disparaging remarks to clients about colleagues or other therapies in the regular or alternative field.

19. We respect and safeguard the professional works and original ideas of others, and give full credit and appropriate citation for such works and ideas when used in our publications and presentations.

20. We are willing to discuss our professional activities with fellow members. We collaborate with complaint procedures.

21. We will assess our own competence, skills and limitations. We seek additional knowledge and skills through participation in seminars. Workshops or trainings and other opportunities. We keep informed about developments in the profession and in society.

22. We will collaborate with any serious research that does not prejudice the therapy process. We adhere to the generally accepted academic guide lines in conducting research with people.

23. We exercise prudence and discretion in promoting, encouraging, and facilitating public awareness and understanding of the field. We avoid sensationalism, inaccuracy or misleading claims.

24. We adhere to professional ly accepted guidelines and standards of good taste in informing the general public of the availability of our services.

25. We seek to protect the public from all those who are not qualified to practice by deficient education, preparation, competence, emotion al stability, or ethical standards.

26. When we observe inappropriate professional practices of fellow members, we will, whenever possible, discuss this with them. If this does not work, we will inform EKAA (CHII) in writing.